جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : عايدة ابراهيم عبد الفتاح عبد القوى

المحافظة : الجيزه **الحي / المدينة :** الصف **العنوان :** الشوبك الشرقى الصف الجيزة

رقم البطاقة : 28509032101181 **رقم التليفون :** 01020323874 **تاريخ تسجيل الإستمارة :** 20-10-2026

العمر(تاريخ الميلاد) : 03-09-1985 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عايدة ابراهيم عبد الفتاح عبد القوى
National ID: 28509032101181

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

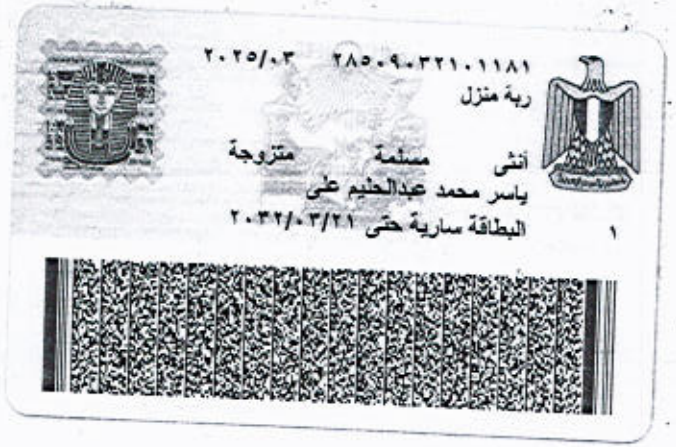
نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Aida Ibrahim Abdel Fattah Abdel Kawy	Date : 04/09/2024
Exam : MAMMOGRAPHY BOTH	Age : 39 Year(s)
Medical ID: 2812/2024	Pat. ID: 78532

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had CBS in left breast.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions. Small seroma collection at operative bed about 44x12mm.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness about4mm.
- Left axilla operative bed seroma about 16x10mm.
- Free left infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Signature Dr. Nehal Hamdy

04/09/2024 2:44 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name: عايدة ابراهيم عبد الفتاح عبد القوي	
MR. : 5481	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient is a 40-year-old premenopausal individual with a history of left breast-conserving surgery performed at an outside institution for invasive ductal carcinoma, grade 2, with nodal involvement (9 of 16 nodes positive). The tumor was hormone-receptor positive (ER 7/8, PR 5/8) and HER2 negative. Post-operative staging CT of the abdomen identified two focal hepatic lesions, which were interpreted as metastatic disease. The patient was initiated on weekly paclitaxel for 24 weeks, followed by maintenance endocrine therapy with goserelin (Zoladex) and letrozole (Femara). Serial imaging demonstrated stable hepatic lesions: CT in September 2024 showed lesions measuring approximately 2.7 × 2.5 cm; CT in May 2025 and November 2025 confirmed that the lesions remained unchanged in size, with no radiologic evidence of progression. A bone scan showed no definite metastatic involvement. The patient continues on goserelin and letrozole.

An MRI of the liver with contrast was ordered to further characterize the hepatic lesions. The patient remained on goserelin and letrozole, and the plan was to continue this regimen for an additional three months while awaiting MRI results.

MRI of the liver performed in June 2026 demonstrated the same two right hepatic focal lesions, measuring 1.7 × 1.5 cm and 1.5 × 2 cm, respectively. The lesions were unchanged from prior imaging, and the recommendation was to follow them with repeat imaging in three months. The patient remains on goserelin and letrozole.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : علياء احمد أبو النصر شهاب الدين

يعتمد ,

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : عايده ابراهيم عبدالفتاح عبدالقوى Sex: Female
3- Medical number: 83227370
Old Med : 2812-2024
2-ID number:
Patient Age : 39 Y

MDT

0 0

Family history	Family history	Phone	Phone 01020323874	BMI	BMI	REASON OF MDT	REASON OF MDT
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT 01/10/2024
Co morbidity	Co Morbidity -ve	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :	Mets Workup : ct:quiry HFL

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	7/8	5/8	0		18
BX COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	

T	N	M
		liver

Sonomammography

PATIENT HISTORY

S/P:Lt BCS outside our hospital , IDC Grade 2
LN 9/16
ct abdomen : 2 hepatic focal lesion mostly metastatic
NB CT abdomen done after the operation

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

start taxol weekly for 3 months then reevaluate by CT
consider maintinance hormonal treatment zoladex +AI AFTER 3 OR 6 month of taxol

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY أ.د/شريف حسانين أ.د/يحيى اسماعيل	RADIOLOGY	RADIOLOGY أ.د/شريف سامح
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

MDT Form
[[DOCTYPE:MDT_FORM]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Aida Ibrahim Abdel Fattah Abdel Kawy	Date : 17/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 40 Year(s)
Medical ID: 2812/2024	Pat. ID: 78532

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- **Compared to last study dated 8-5-2025.**

I Chest:

- No sizable suspicous pulmonary nodules, masses.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- Still seen left axillary region soft tissue sheet inseparable from the pectoralis major muscle likely post operative sequel.
- Still noted right axillary lymph nodes.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- The liver still showed two right lobe segment V/VI and VII enhancing focal lesions, no newly developed lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, right suprarenals gland, aorta and IVC.
- Stationary left adrenal nodule measuring 1.5 x 1.2 cm.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

- ***Stationary course since the last study.***

Dr. Asser Ahmed

Dr. Asser Ahmed

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name	عايده ابراهيم عبد الفتاح	Code Number	YK 24021917
Sex/Age	Female 40 Y	Patient ID	26394
Registered Date	10/8/2024 06:27:41 PM	Referred By Dr.	ابراهيم عبد الرحمن
Validation Date	17/8/2024 02:02:04 PM	Branch	الفرع الرئيسي الدقى
Reporting Date	17/8/2024 02:05:15 PM		

Clinical Diagnosis: Left Breast Mass
Nature of the specimen: Wide Local Excision And Left Axillary Lymph Node Dissection

Pathology Report

Gross Picture

Two designated specimens received:
A) An irregular fibro fatty tissue piece measured 8x7x4.5 cm, attached skin ellipse measured 4.3x2 cm. (marked by sutures). Serial cuts revealed ill-defined mass measured 2.8x2x2 cm with firm gritty gray, tan cut section located (1.5 cm, 1.3 cm, 2 cm, 2.3 cm, 0.5 cm, 2 cm) from (long suture, opposite to long suture, short suture, opposite to short suture, deep, skin) margins. Also detected fairly defined area measured 2x1.8 cm with soft fleshy cut section.
B) An irregular fibro fatty tissue piece measured 8x8 cm collectively, entangling 16 nodules measured up to 2.7x1.8 cm with firm gritty gray, white cut section.

Microscopic Picture

(A&B) Sections examined from breast tissue revealed malignant neoplastic growth formed of large ductal cells arranged in sheets and irregular ducts, showing moderate degree of anaplasia with hyperchromatic pleomorphic nuclei within wide sclerotic desmoplastic stroma. There is marked tumor infiltrating lymphocytes. Tumor vascular emboli and perineural invasion not seen. No DCIS component seen. The surgical margins were free of tumor involvement. Sections examined from dissected lymph nodes revealed macro metastasis in 9 out of 16 nodes with capsular invasion and perinodal fat infiltration (9/16).

Scan Me

Patient Name	عايده ابراهيم عبد الفتاح	Code Number	YK 24021917
Sex/Age	Female 40 Y	Patient ID	26394
Registered Date	10/8/2024 06:27:41 PM	Referred By Dr.	ابراهيم عبد الرحمن
Validation Date	17/8/2024 02:02:04 PM	Branch	الفرع الرئيسي الدقى
Reporting Date	17/8/2024 02:05:15 PM		

Diagnosis

(A&B) Left Broast Mass, Wide Local Excision And Left Axillary Lymph Node Dissection:
Invasive Breast Cancer Of NST (Invasive Duct Carcinoma) Grade II, PT2 N2.
Size Of Invasive Carcinoma: 2.8x2 cm.
Marked Tumor Infiltrating Lymphocytes.
No Associated DCIS Components.
No Evident Peritumoral Lymphovascular Emboli Or Perineural Invasion.
Resection Margins Status: Free Of Tumor Involvement.
Axillary Lymph Nodes Status: Positive For Macro Metastasis (9/16) With Capsular Invasion And Perinodal Fat Infiltration.

P.O

Number of slides enclosed: 12

Diagnosed by

Dr/ Amr AL-Amaragy

Dr/ Mona Sakr

Scan Me

??????, ????? **Patient ID: 10110224013469 DOB: 03-Sep-1985 Date: 17-Nov-2024**
Patient Lastname: ابراهيم
Patient Firstname: عايده
HIS Patient ID: 10110224013469
RIS Patient ID: 362073
Date of Birth: 03/09/1985

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========
99mTc MDP Bone Scan Report (17/11/2024)
* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
" Degenerative changes are noted over the dorso-lumbar vertebrae.
" Arthritic changes are noted over both shoulder and both knee joints.
" Both kidneys are visualized.
Conclusion:
" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged
Omnia Talaat, MD

========== REPORT TEXT ENDE ==========